

MEDSHIELD CLINICAL MEDICAL CENTER

OFFICIAL INPATIENT CLINICAL CHART & DISCHARGE SUMMARY

Patient Name: John A. Smith	DOB: March 15, 1965 (Age 61)
MRN: MRN-2026-04821	Gender: Male Blood Type: A+
Admission Date: August 15, 2026	Discharge Date: August 22, 2026
Department: Cardiology (CCU)	Attending: Dr. Sarah Chen, MD (Cardiology)
Primary Diagnosis: Acute STEMI (ICD-10: I21.0)	Insurance: Medicare Part B

1. CHIEF COMPLAINT & HISTORY OF PRESENT ILLNESS

65-year-old male presenting to the Emergency Department with acute onset substernal chest pressure radiating to left arm and jaw, diaphoresis, nausea, and dyspnea starting 3 hours prior to arrival while mowing lawn. Pain rated 8/10. Emergent coronary angiography revealed 95% occlusion of the proximal Left Anterior Descending (LAD) artery. Successful percutaneous coronary intervention (PCI) with drug-eluting stent placement was performed.

2. PAST MEDICAL HISTORY & COMORBIDITIES

- Type 2 Diabetes Mellitus** (Diagnosed 2018) — Baseline HbA1c elevated at 7.8%
- Essential Hypertension** (Diagnosed 2015) — Managed with ACE inhibitor
- Hyperlipidemia** (Diagnosed 2016) — History of mixed dyslipidemia
- Social History:** Former smoker, 30 pack-year history, quit in 2021. Denies illicit drug use.
- Allergies:** No Known Drug Allergies (NKDA).

3. VITAL SIGNS (ADMISSION vs. DISCHARGE)

Parameter	On Admission (Aug 15)	At Discharge (Aug 22)	Reference Range
Blood Pressure	158/95 mmHg (Hypertensive)	122/78 mmHg (Optimal)	90-120 / 60-80 mmHg
Heart Rate	102 bpm (Tachycardic)	68 bpm (Normal sinus rhythm)	60-100 bpm
SpO2	94% on Room Air	99% on Room Air	95-100%
BMI	30.1 kg/m2 (Obese Class I)	30.1 kg/m2	18.5-24.9 kg/m2

4. CLINICAL LABORATORY PANELS

Test Name	Result	Standard Unit	Reference Interval	Clinical Flag
Troponin I (Peak)	14.80	ng/mL	0.00 - 0.04	CRITICAL HIGH
HbA1c (Glycated Hb)	7.80	%	4.00 - 5.60	ELEVATED
Fasting Blood Glucose	195.00	mg/dL	70.00 - 100.00	ELEVATED
Total Cholesterol	265.00	mg/dL	0.00 - 200.00	HIGH

LDL Cholesterol	168.00	mg/dL	0.00 - 100.00	HIGH
HDL Cholesterol	38.00	mg/dL	40.00 - 60.00	LOW
Serum Creatinine	1.10	mg/dL	0.70 - 1.30	NORMAL
eGFR	78.00	mL/min/1.73m2	> 60.00	NORMAL

5. DISCHARGE MEDICATIONS & REGIMEN

Medication	Dosage	Frequency	Indication / Duration
Aspirin	81 mg	Once daily	Antiplatelet therapy (Indefinite)
Ticagrelor (Brilinta)	90 mg	Twice daily	DAPT post-PCI (Minimum 12 months)
Atorvastatin (Lipitor)	80 mg	Once daily at bedtime	High-intensity statin therapy
Metoprolol Succinate	50 mg	Once daily	Post-MI cardioprotection
Lisinopril	10 mg	Once daily	Blood pressure & ventricular remodeling
Metformin	500 mg	Twice daily with meals	Type 2 Diabetes glycemic control

6. CLINICAL DISCHARGE PLAN & FOLLOW-UP SCHEDULE

- **Cardiology Follow-Up:** Appointment with Dr. Sarah Chen in 2 weeks (Sept 5, 2026) for repeat ECG and echocardiogram.
- **Cardiac Rehabilitation:** Outpatient phase II cardiac rehab referral submitted; start in 3 weeks.
- **Endocrinology / Primary Care:** Follow-up in 4 weeks to repeat HbA1c and lipid panel; consider uptitrating Metformin if HbA1c remains > 7.0%.
- **Warning Signs / Red Flags:** Return to ED immediately if recurrent chest pressure, dyspnea at rest, palpitations, or presyncope occurs.